

Streamlining Copay Assistance for CIDP & Myasthenia Gravis: Rapid Foundation Enrollment via GetCopayHelp

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BACKGROUND: High OOP Burden in Neuro Care—IVIG & Complement Rx for CIDP/MG > \$8 K

Manufacturer copay cards run on established claims rails, but diagnosis-based foundations remain a digital blind spot for both IV and oral drugs. Each portal has unique rules and unpredictable openings, and funds can fill within a day. Manual “refresh fund portals → download → collect income info → fax/upload” cycles take one-to-two days—long enough for funds to close and patients to face steep costs. Coordination is heavier than with manufacturer programs, so missed grants trigger infusion delays or abandonment. Rising Alzheimer’s anti-amyloid volumes will amplify this load.

OBJECTIVE:

Evaluate a standalone AI “search→match→enroll” module—fed by simple intake methods—to coordinate with patients, and secure foundation grants in the short fund-open window and curb therapy drop-offs, without bundling manufacturer, copay or PAP, tools.

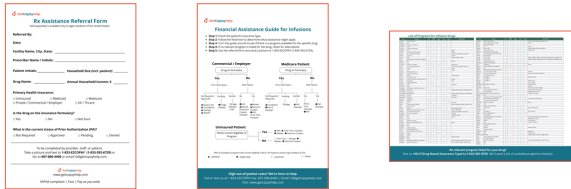
METHODS:

Prospective, single-site quality-improvement project in a neurology clinic (seven infusion chairs)

	Title	Concise description
1	External “Foundation Assistance Concierge.”	Practice offloads all foundation tasks to GetCopayHelp; we take over patient interaction to complete Foundation Assistance enrollments
2	Frictionless Program Search & Intake.	Platform polls funds, auto-matches patients; accepts email (EHR exports), or the one-page MMS form (see note).
3	Same-Day Patient Engagement & Submission.	GetCopayHelp gathers docs and with AI-enabled automations files apps the same day a fund opens, aiming to keep therapy on schedule.

PAPER TO CUT CLICKS

An experimental pilot of a one-page checklist: staff snap & send via email/MMS; OCR pulls data, matches funds, and emails (or EHR-drops) a pre-filled packet—no extra portal required.



EVALUATION PLAN (Feb 2025 – Jul 2026):

- **Speed & workload:** % of eligible patients funded ≤ 24 h after a fund opens, median hours to grant, and staff minutes per enrollment (manual vs GetCopayHelp).
- **Brand flexibility & therapy persistence:** evaluate how real-time visibility into alternative, assistance-supported brands sustains treatment.
- **Low-friction intake:** gauge whether a one-page checkbox form reduces digital overload by removing extra portals for staff.

ANTICIPATED IMPACT:

A lightweight, drop-in module like GetCopayHelp secures foundation grants the same day they open, keeping patients on therapy through brand-flexible options. A single checkbox intake eliminates new portals or workload and coexists seamlessly with any existing manufacturer cards or PAP tools.